

Incident Handler's Journal

Fictional scenario. Ongoing journal maintained during the Google Cybersecurity Professional Certificate, "Sound the Alarm: Detection and Response" course. Additional entries are appended as later course activities are completed.

Entry 1

Date	September 6, 2026
Case ID	INC-2026-0091
Description	Ransomware incident, primary-care clinic (single site) — initial intake from IT/end-user reports.
Tool(s) used	None yet. Logged from initial IT/end-user reports via ticketing system. Anticipated for investigation: EDR console, email gateway logs, SIEM.
The 5 W's	Who: Unidentified external threat actor. Ransom note and encryption behavior suggest a RaaS affiliate rather than the malware's original developer. Attribution unconfirmed. What: Employee opened a phishing attachment, installing malware. Attacker used the foothold to move laterally and deploy ransomware, encrypting files needed for clinical operations. Ransom note demanding payment for a decryption key left on affected machines. When: Reported ~9:00 a.m. Tuesday. Phishing delivery/execution time not yet confirmed; pulling email gateway timestamps next. Where: Employee workstations and patient-record systems at the clinic's single location. Full scope and network segmentation not yet confirmed. Why: Attacker's motive is financial extortion; healthcare is a common target because operational disruption creates pressure to pay quickly. Deployment succeeded because the phishing email reached and was executed by a user, the malware established a foothold, and it was able to reach and encrypt clinical systems from there. Exact technical path (initial execution to encryption) still to be traced.
Additional notes	HIPAA breach notification likely applies — flag to compliance/legal. Exfiltration not yet ruled out (double-extortion is common); would raise severity if confirmed. Backup availability unknown — check immediately, determines recovery path. Next: preserve evidence, isolate affected systems, secure phishing email for analysis, engage FBI IC3 and external IR support.